



Loss, Grief and Death

Loss: is an actual or potential situation in which something that is valued is changed or no longer available.

- Individuals can experience the loss of body image, a significant other, a sense of well-being, a job.
- Illness and hospitalization often produce losses.

Death: is a loss both for the dying individual and for those who survive.

Although death is inevitable, it can stimulate individuals to grow in their understanding of themselves and others.

Individuals experiencing loss often search for the meaning of the event, and it is generally accepted that finding meaning is needed for healing to occur.

- **Grief:** is the total response to the emotional experience related to loss.

Grief is manifested in thoughts, feelings, and behaviors associated with sorrow.

• Types of Loss

- ♦ An **actual loss** can be recognized by others.
- ♦ A **perceived loss** is experienced by an individual but cannot be verified by others.

Sources of loss

1. loss of an aspect of oneself—a body part, a physiologic function, or a psychologic attribute
2. loss of an object external to oneself
3. separation from an accustomed environment
4. loss of a loved or valued individual.

Types of grief

Abbreviated grief

- It is brief but genuinely felt. This can occur when the lost object is not significantly important to the grieving individual or may have been replaced immediately by another, equally esteemed object.

Anticipatory grief

- It is experienced in advance of the event such as the wife who grieves before her ailing husband dies.
- A young individual may grieve before an operation that will leave a scar. Because many of the normal symptoms of grief will have already been expressed in anticipation, the reaction when the loss actually occurs is sometimes quite abbreviated.

Disenfranchised grief

- occurs when an individual is unable to acknowledge the loss to others.
- Situations in which this may occur often relate to a socially unacceptable loss that cannot be spoken about, such as suicide, abortion, or giving a child up for adoption.

Unhealthy grief

- Pathologic or complicated grief—exists when the strategies to cope with the loss are maladaptive and out of proportion or inconsistent with cultural, religious, or age appropriate norms.

Stages of Grieving

Denial

Behavioral Responses:

- Refuses to believe that loss is happening.
- Is unready to deal with practical problems, such as prosthesis after the loss of a leg.

Nursing Implications

- - Verbally support client but do not reinforce denial.

Anger

- **Behavioral Responses**

Client or family may direct anger at nurse or staff about matters that normally would not bother them.

- **Nursing Implications**

- Help client understand that anger is a normal response to feelings of loss and powerlessness.

- Avoid withdrawal; do not take anger personally.

- Deal with needs underlying any angry reaction.

- Allow clients as much control as possible over their lives.

Bargaining

- **Behavioral Responses**

Seeks to bargain to avoid loss (e.g., “let me just live until [acertain time] and then I will be ready to die”).

- **Nursing Implications**

Listen attentively and encourage client to talk to relieve guilt and irrational fear. If appropriate, offer spiritual support.

Depression

- **Behavioral Responses**

Grieves over what has happened and what cannot be. May talk freely (e.g., reviewing past losses such as money or job)

- **Nursing Implications**

- Allow client to express sadness.
- Communicate nonverbally by sitting quietly without expecting conversation.
- Convey caring by touch.

Acceptance

- **Behavioral Responses**

- May have decreased interest in surroundings and support people.
- May wish to begin making plans (e.g., will, prosthesis, altered living arrangements).

- **Nursing Implications**

- Help family and friends understand client's decreased need to socialize.
- Encourage client to participate as much as possible in the treatment program.

Manifestations of Grief

- ♦ **Manifestations of grief considered normal include verbalization of the loss, crying, sleep disturbance, loss of appetite and difficulty concentrating.**
- ♦ **Complicated grieving may be characterized by extended time of denial, depression, severe physiologic symptoms, or suicidal thoughts.**

Significance of the Loss

- ♦ The significance of a loss **depends on the perceptions of the individual experiencing the loss**. One individual may experience a great sense of loss over a divorce; another may find it only mildly disrupting.
- ♦ Several **factors affect the significance of the loss**: Importance of the lost individual, object, or function, degree of change required because of the loss, the individual's beliefs and values.

Factors Influencing the Loss and Grief Responses

□ Age

- ♦ Age **affects** an individual's **understanding of and reaction to loss.**
Individuals usually increase their understanding and acceptance of life, loss, and death.

□ Culture

- ♦ **Culture influences an individual's reaction to loss.**
- ♦ **How grief is expressed is often determined by the customs of the culture.**
- ♦ **Family structure affect on perception of grief.**

Example: The death of a family member in a typical nuclear family leaves a great void because the same few individuals fill most of the roles.

In cultures where several generations and extended family members either reside in the same household or are physically close, the impact of a family member's death may be softened because the roles of the deceased are quickly filled by other relatives. Some individuals believe that grief is a private matter to be endured internally.

□ Spiritual Beliefs

- ♦ Spiritual beliefs and practices greatly influence both an individual's reaction to loss and subsequent behavior.
- ♦ Most religious groups have practices related to dying, and these are often important to the client and support people.
- ♦ To provide support at a time of death, nurses need to understand the client's particular beliefs and practices.

□ Gender

- ♦ **Males are frequently expected to “be strong” and show very little emotion during grief, whereas it is acceptable for females to show grief by crying.**
- ♦ **Gender roles also affect the significance of body image changes to clients.**

Example: A man might consider his facial scar to be “macho,” but a woman might consider hers ugly. Thus the woman, but not the man, would see the change as a loss.

□ Socioeconomic Status

- ♦ The socioeconomic status of an individual often **affects the support system available at the time of a loss.**

Example: A pension plan or insurance, for example, can offer an individual who is widowed or disabled a choice of ways to deal with a loss; an individual who is confronted with both severe loss and economic hardship may not be able to cope with either.

□ Support System

- ♦ **The individuals closest to the grieving individual are often the first to recognize and provide needed emotional, physical, and functional assistance.**

Dying and Death

Definition of Death

- **The traditional clinical signs of death were cessation of the apical pulse, respirations, and blood pressure, also referred to as heart-lung death.**
- **Another definition of death is cerebral death or higher brain death**

Death-Related Religious and Cultural Practices

- ♦ **Cultural and religious traditions and practices** associated with death, dying, and the grieving process **help clients cope with these experiences.**
- ♦ **Knowledge of the client's religious and cultural heritage helps nurses provide individualized care to clients and their families,**

Examples: Some individuals prefer a peaceful death at home rather than in the hospital.

- ♦ Members of certain ethnic groups may request that health professionals not reveal the prognosis to dying clients. They believe the individual's last days should be free of worry.
- ♦ Other cultures prefer that a family member (preferably a male in some cultures) be told the diagnosis so the client can be tactfully informed by a family member in gradual stages or not be told at all.

Death-Related Legal Issues

- ♦ **Laws** that describe issues involving decisions about death and dying are constantly changing.
- ♦ These include advance directives, do not resuscitate, organ donation, and aid in dying.
- ♦ Nurses must remain knowledgeable about the legal issues.

Advance Healthcare Directives

- ♦ It is legal document that describes preferences for future treatment, whether or not the client is currently unwell.
- ♦ **Examples: do not resuscitate, organ donation, and aid in dying.**
- ♦ **The client specifies one or more individuals who will serve as their proxy (substitute) in making healthcare decisions should they be unable to do so.**

Do-Not-Resuscitate Orders

- ♦ **Do-not-resuscitate orders**, also referred to as **DNR** refer to the documentation of the decision to refrain from cardiopulmonary resuscitation (CPR) should the client's heart or breathing cease from an irreversible underlying condition.
- ♦ The decision should be made with the client and family, when possible, and always reflect the competent client's wishes.

Organ Donation

- ♦ Both in the U.S. and countries in the EU, **the law allows competent adults to pre-authorize the donation of their organs for research, education, or transplantation.**
- ♦ There are **two main approaches to organ donation: presumed consent and explicit consent.**

Main approaches to organ donation

- ♦ **Presumed consent:** is a legislative framework in which **citizens must place their name on a national opt-out register, otherwise their consent for donating their organs will be presumed.**
- ♦ **Explicit consent:** means that an **individual will not be an organ donor unless he or she explicitly states otherwise.**

Nursing Management

Assessment

- ♦ The nurse first needs to **recognize the states of awareness manifested by the client and family members.**
- ♦ In cases of terminal illness
- ♦ Three types of awareness that have been described are **closed awareness, mutual pretense, and open awareness.**

Types of awareness in loss and death

- ♦ **Open awareness:** the client and others know about the impending death and feel comfortable discussing it, even though it is difficult. This awareness provides the client an opportunity to participate in planning funeral arrangements.
- ♦ **Closed awareness:** the client is not made aware of impending death. The family may choose this because they do not completely understand why the client is ill or they believe the client will recover.

- ♦ **Mutual pretense:** the client, family, and healthcare personnel know that the prognosis is terminal but do not talk about it.

Signs of Impending Clinical Death

Loss of muscle tone

- **Relaxation of the facial muscles (e.g., the jaw may sag)**
- **Difficulty speaking**
- **Difficulty swallowing and gradual loss of the gag reflex**
- **Decreased activity of the gastrointestinal tract, with subsequent nausea, accumulation of flatus, abdominal distention, and retention of feces, especially if narcotics or tranquilizers are being administered.**
- **Possible urinary and rectal incontinence due to decreased sphincter control**
- **Diminished body movement**

❑ Slowing of the circulation

- **Diminished sensation**
- **Mottling and cyanosis of the extremities**
- **Cold skin, first in the feet and later in the hands, ears, and nose**
(the client, however, may feel warm if there is a fever)
- **Slower and weaker pulse**
- **Decreased blood pressure**

- **Changes in respiration**

- **Rapid, shallow, irregular, or abnormally slow respirations**
- **Noisy breathing, referred to as the death rattle, due to collecting of mucus in the throat**
- **Mouth breathing, dry oral mucous membranes**

- **Sensory impairment**

- **Blurred vision**
- **Impaired senses of taste and smell**

• Diagnoses

Diagnoses that may be particularly appropriate for the dying client are fear, hopelessness, and powerlessness.

• Planning

❑ Major goals for dying clients are:

(a) Maintaining physiologic and psychologic comfort

(b) Achieving a dignified and peaceful death, which includes maintaining personal control and accepting declining health status.

• **Implementation**

The major nursing responsibility for clients who are dying is to assist the client to a peaceful death. More specific responsibilities include the following:

- **To minimize loneliness, fear, and depression**
- **To maintain the client's sense of security, self-confidence, dignity, and self-worth**
- **To help the client accept losses**
- **To provide physical comfort.**

- **How to help clients die with dignity and peaceful death**

- Dying individuals often **strive for self-fulfillment more than for self-preservation**, Part of the nurse's challenge is to support the client's will and hope.
- Nurses need to ensure that the client is treated with honor and respect consistent with their values, beliefs, and culture.
- Establish a communication relationship that shows concern for and commitment to the client.
- Make time to be available to the client to provide support, listen, and respond.
- Identify patient's feelings about death and how they may influence interactions with clients.
- Talk to the client or family members about how the client usually copes with stress.

- **Nurses can help clients to determine their own physical, psychologic, and social priorities.**
- **Meeting the physiologic needs include providing personal hygiene measures; controlling pain; relieving respiratory difficulties; assisting with movement, nutrition, hydration, and elimination; and providing measures related to sensory changes**
- **Restore and support feelings of control. Some choices that clients can make are the location of care (e.g., hospital, home, or hospice facility), times of appointments with health professionals, activity schedule, and times of visits from relatives and friends.**

- **Acknowledge personal fears about death and discuss them with a friend or colleague.**
- **Provide a caring touch.** Holding the client's hand or offering a comforting massage can encourage the client to verbalize feelings.
- **Respond with honesty and directness to the client's questions about death.**
- **Providing spiritual support** either through direct intervention or by arranging access to individuals who can provide spiritual care.

- **Supporting the Family involve using therapeutic communication to facilitate their expression of feelings.**
- **The nurse can provide an empathetic and caring presence. The nurse also serves as a teacher, explaining what is happening and what the family can expect.**

Hospice and Palliative Care

- Hospice care: focuses on support and care of the dying client with a life expectancy of 6 months or less and the family, with the goal of facilitating a peaceful and dignified death.
- Palliative care: through the prevention and relief of suffering by means of early identification

• Postmortem Care

- **Rigor mortis:** is the stiffening of the body that occurs about 2 to 4 hours after death.
- **Rigor mortis** starts in the involuntary muscles (heart, bladder, and so on), then progresses to the head, neck, and trunk, and finally reaches the extremities.

- **Algor mortis:** is the gradual decrease of the body's temperature after death. When blood circulation terminates and the hypothalamus ceases to function, body temperature falls about 1°C (1.8°F) per hour until it reaches room temperature.

Simultaneously, the skin loses its elasticity and can easily be broken when removing dressings and adhesive tape. After blood circulation has ceased, the red blood cells break down, releasing hemoglobin, which discolors the surrounding tissues.

- **This discoloration, referred to as appears in the lowermost areas of the body.**
- **Tissues after death become soft and eventually liquefied by bacterial fermentation.**
- **The hotter the temperature, the more rapid the change. Therefore, bodies are often stored in cool places to delay this process. Embalming prevents the process through injection of chemicals into the body to destroy the bacteria.**

- **Nursing care of a body after death.**

- **Postmortem care should be carried out according to the policy of the hospital or agency.**
- **Because care of the body may be influenced by religious law, the nurse should check the client's religion.**
- **If the deceased's family or friends wish to view the body, make the environment clean and pleasant and make the body appear natural and comfortable.**

- Normally the body is placed in a supine position with the arms either at the sides, palms down, or across the abdomen. One pillow is placed under the head and shoulders to prevent blood from discoloring the face by settling in it. The eyelids are closed.
- The mouth is then closed. Soiled areas of the body are washed.
- Absorbent pads are placed under the buttocks to take up any feces and urine released because of relaxation of the sphincter muscles. A clean gown is placed on the client, and the hair is arranged. All jewelry is removed, except a wedding band in some instances, which is taped to the finger.
- The top bedlinens are adjusted neatly to cover the client to the shoulders. After the body has been viewed by the family, the deceased's wrist identification tag is left on and additional identification tags are applied.

- **All equipment, soiled linen, and supplies should be removed from the bedside.** Some agencies require that all tubes in the body remain in place; in other agencies, tubes may be cut to within 2.5 cm (1 in.) of the skin and taped in place; in others, all tubes may be removed.

- **The body is wrapped in a shroud, a large piece of plastic or cotton material used to enclose a body after death. Identification is then applied to the outside of the shroud.**
- **Nurses have a duty to handle the deceased with dignity and to label it appropriately.**

- **Evaluating**

- **To evaluate the achievement of client goals, the nurse collects data in accordance with the desired outcomes established in the planning phase. Evaluation activities may include the following: Listening to the client's reports of feeling in control of the environment surrounding death, such as control over pain relief, visit of family, support people, and treatment plans.**